Colegio International de Caracas

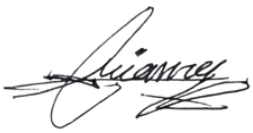
STUDY GUIDE

JMUN 2026

WHO

1. Introduction

Honorable Delegates, It is our distinct honor to welcome you to the World Health Organization (WHO) at JMUN 2026. First and foremost, we congratulate you on taking the initiative to participate in this conference. Through this event, you will have the opportunity to refine your public speaking, debating, and negotiation skills while simulating the work of one of the world's most vital international organizations. Beyond the academic rigor, you will have the opportunity to forge alliances that may turn into lifelong friendships. As your Dais, we are committed to giving our absolute best to ensure this experience is both educational and inspiring. The topic for the World Health Organization this year will be: "International Response to the Ebola Epidemic." We look forward to a session filled with enriching and cooperative debate. Sincerely,



Ariana Azuma, Chair

Isabella Ojeda, Co-Chair

Letter from Chair:

My name is Ariana Azuma, and it is my absolute pleasure to serve as your Chair. My journey in Model United Nations began much like yours, starting as a delegate in JMUN. Since then, I have dedicated my time to understanding the mechanics of how a great committee runs. While I have participated as a delegate in various conferences, I found my true passion on the Dais. I have had the privilege of serving as a Co-Chair in several prestigious events, including SAMUN and previous JMUN sessions. My experience behind the gavel has taught me how to spot a strong argument, how to guide a room toward consensus, and most importantly how to ensure every delegate, from the most experienced to the first-timer, feels empowered to speak. I promise to lead with professionalism and respect. My goal is to facilitate a committee that is academically rigorous but also deeply enjoyable. I am incredibly excited to lead this session and to see the world-changing ideas you all bring to the floor.

Letter from Co-Chair

Distinguished delegates,myname is Isabella Ojeda. I am a ninth-grade student at the Colegio Internacional deCaracas. It is my pleasure to be your co-chair for this session of JMUN 2025. I startedoutas a member of the Model United Nations, with JMUN as my first event. So far,Ihave participated in JMUN 2024 (Colegio Internacional de Caracas), CLAMUN (Colegio Claret), and CIAMUN (Colegio Integral el Avila). I have also been an observer in several VIMUN committees (Ciudad Banesco), winning awards such as several honorable mentions and verbal mentions. I have several ideas about how this committee will work. This being my second experience participating on the board and also on the World Health Organization session side, I promise that it will be an experience carried out with professionalism and the utmost respect. This will make it an enjoyable experience for the delegates. This committee will hold discussions and come up with solutions, but everything will be handled in the best possible way so that there are no disagreements and the committee runs smoothly. I am very excited and happy to be part of this great experience and to be able to lead this delegation. With nothing more to say, Welcome to the Committee!

2. Matrix

COMMITTEE MATRIX
Guinea
Liberia
Sierra Leone
Democratic Republic of Congo
Russia
United kingdom
India
China
Cuba
Nigeria
Mexico
Indonesia
Saudi Arabia
USA
Switzerland

3. Introduction to the Committee

The committee of the WHO or the World Health Organization is a United Nations specialized agency composed of 194 member states that aims to promote health, safety by enhancing the connections between countries, partners, and people to scientific evidence they can rely on, therefore assisting in the controlling of epidemic and endemic diseases. The World Health organization is the largest intergovernmental health organization internationally governed by the World Health Assembly, that is in charge of coordinating the responses to international public health issues and emergencies. Similarly, it provides technical assistance to nations, dictates standards and guidelines for international health and assembles data regarding global health problematics. This committee wants to achieve that "*everyone, everywhere can attain the highest level of health".* (Ghebreyesus) Sovereign states are the only countries that are considered eligible to join to his committee. The formation of the WHO was on 7 April 1948, officially operating later on that year on september 1st. Originally, the WHO was designed to consolidate global medical efforts, inheriting the essential responsibilities, staff, and assets of both the League of Nations' Health Organization and the Office International d'Hygiène Publique. The transition that involved the management of the International Classification of Diseases (ICD) opened a cornerstone of global health today. The WHO has demonstrated the power of international cooperation when succeding in eradicating smallpox and its current progress towards a polio free world. Recently this organization has focused on adressing the problematic of the Ebola virus, aiming to work in vaccine research and emergency response protocols. This demonstrates us that the WHO does not only fight viruses but work on pushing for healthcare for everyone, leading the world's response to emergencies, the WHO acts like a global shield. It protects us from sudden outbreaks ike the case of Ebola and from long-term health problems that affect people every day."(Internews)

4. Introduction to the topic

Ebola virus disease is a severe and often fatal viral hemorrhagic fever (average fatality rate of 50%) that affects humans and non-human primates. It is transmitted through direct contact with the bodily fluids of infected people or animals, and its symptoms include high fever, diarrhea, vomiting, and internal bleeding.

It was discovered in 1976 near the Ebola River in the Democratic Republic of Congo (DRC), Ebola virus disease has caused outbreaks mainly in Africa, with small, isolated episodes in

other countries. Confirmed cases outside Africa include Nigeria, Senegal, Mali, Spain, the United States, the United Kingdom, and Italy. The West African epidemic (2014-2016) was the largest on record: it began in Guinea in December 2013, spread rapidly to Liberia and Sierra Leone, and caused more than 28,600 cases and 11,300 deaths (case fatality rate ~50%). Aggravating factors included extreme poverty, weak health systems, and local funeral practices that facilitated its spread to densely populated urban areas.

5. Key terms

Intergovernmental Organization(Igo): An organization made up of different countries (Member States). The WHO is an IGO because it is a place where governments work together, rather than a private company or a single country's project.

World Health Assembly (WHA): The "decision-making" body of the WHO. Think of it as the "Parliament" of health. It is where all 194 Member States meet to set policies, appoint leaders, and approve the budget.

Soverign State: A country that has its own government and the power to make its own laws and decisions. Only independent, recognized countries can be full members of the WHO.

Epidemic: A sudden increase in the number of cases of a disease (like an Ebola outbreak) that spreads quickly through a population.

Endemic: A disease that is always present in a certain area (like malaria in some tropical regions).

International classification of diseases (ICD): The "Global Rulebook" for health. It is a system that gives every disease a specific code. This allows doctors in Venezuela to talk to doctors in Japan and know they are talking about the exact same illness.

Eradication: The permanent reduction to zero of a disease worldwide. The WHO’s greatest victory was the eradication of Smallpox, meaning the virus no longer exists in the wild.

Universal Health Coverage (UHC): The goal is that all people can get the health services they need (prevention, treatment, and pain management) without suffering financial hardship or going into debt.

Emergency Response Protocols: A set of pre-planned rules and steps that countries and doctors must follow as soon as a disease starts to spread. It's like a "fire drill" for a virus to ensure no time is wasted.

Technical Assistance: When the WHO sends experts, tools, or training to a country to help them improve their hospitals or labs, rather than just sending money.

6. History of the topic

The discovery of the Ebola virus in 1976 represented one of the most shocking milestones in modern virology, originating from two almost simultaneous outbreaks of an unknown and highly lethal hemorrhagic fever in Nzara (Sudan) and Yambuku (Zaire). The pathogen was named after the Ebola River, near the Zairean village where an international team of scientists, led by key figures such as Belgian microbiologist Peter Piot, managed to isolate the virus after investigating a devastating chain of infections that spread through contact with bodily fluids, traditional funeral rites, and the accidental use of unsterilized needles in local hospitals. Biologically, researchers determined that this microorganism belongs to the Filoviridae family, characterized by its unusual thread- or rod-like morphology—similar to the Marburg virus—and has since set a precedent in global public health due to its extreme mortality rate and the technical complexity required to contain its transmission in resource-limited settings.

7. Discussion of the topic

1. Deep distrust of governments and burial restriction conflicts

Research from “The Lancet” and “NIH” has shown that decades of civil war conflict have left many West Africans with a pervasive mistrust (PMC) of state institutions or state authority. Additionally, this problematic was accompanied by the emergence of burial mandates. In regions like Liberia, because of cultural norms, people began burying the dead as a secret, as an effort to avoid cremations. Consequently, these hidden ceremonies accounted for about 20% of all Ebola transmissions. In many cultures, washing or touching the body of a loved one is a sacred ritual, but it's also how diseases like Ebola spread. When officials try to stop these traditions, it causes anger and even violence. To fix this, health workers have to work with local leaders to find a way to honor the dead safely while keeping the living protected. (“Global distribution and health impact of infectious disease outbreaks, 1996–2023: a worldwide retrospective analysis of World Health Organization emergency event reports”)

2. Global Health Inequalities

SincetheoriginsoftheEbolaepidemic in 2014, a ethical controversy has surrounded the use and location of resources. Disparity in resources has been pointed out as they leave low and middle-income countries disproportionately vulnerable to various infectious threats. This disparity is reflected in the unequal distribution of essential diagnostic tools, vaccines, and specialized medical equipment. Which points out a long history of neglect in lots of regions. (nature scientific reports) The issue of non-compliance with international health law and the lack of political commitment have been manifested until the virus was considered by the Western security as a major threat. Experts revealed the disparity in access to experimental treatments and medical evacuations for Westerners versus the local population. (Ogata and Sen)

There is a noticeably difference between rich and poor countries regarding healthcare. Wealthy nations have plenty of doctors, medicine, and vaccines. In contrast, poorer nations struggle to have the basic needs. This is unfair and dangerous for everyone. Viruses, epidemics, and all pathogens don't care about borders, this is why a small outbreak in a place with a weak health system can quickly turn into a global problem. These inequalities create

weak stability in global health security, demonstrating how it will not be safe if it has fragile healthcare systems. (PMC)

3.Sociopoliticalandeconomicconditions

As previously stated, the civil wars had left several countries in terrible economic conditions. It is important to know that infectious outbreaks such as the Ebola epidemic are not simply medical events, but deeply bonded problematics with a nation's sociopolitical and economic stability. For instance, high levels of poverty, food insecurity and protracted conflict simplify the opportunity to get sick and the capacity for a country to provide basic services like funding healthcare. Countries such as Guinea, Liberia, and Sierra Leone are true examples of this. This denotes how poverty and having a weak infrastructure acted as catalysts for the Ebola Epidemic's spread. ("Future Issues in Global Health: Challenges and Conundrums") Yet it demontrates also how as an alternate or logical effects, the epidemics can ruin a countries economy through disrupted trade, lost labor, and limiting resources, affecting mainly vulnerable communities. It is recognized as a vicious cycle *"being poor makes the disease worse, and the disease makes people even poorer".*

4.FutureoutbreaksinWestAfrica

Besides the Ebola epidemic, the West africa has remained as a high risk zone for emerging infectious diseases, specially zoonotic "spillovers" in which pathogens are transmitted from animals to humans because of the increasing urbanization and environmental changes. This problematic reflects the threat of re-emerging viruses. Due to this, all countries to focus on the need for developing long term investments and surveillance, along new vaccines and the coalition for Epidemic Preparedness Innovation, thus sustained international investment is required. This will noticebly help preventing future large-scale catastrophes in West Africa, and world wide. (Oleribe)

5. Capacity to address infectious diseases

The global response system is a pivotal point that has to be addressed in the committee when talking about the Ebola epidemic. Even though in 2014 West africa established public health emergency operations centres within improved laboratory networks these countries still depend on external funding and international organizations like the World Health Organization for large scale responses. (Oleribe) Nations need their own funding and enough

trained healthcare workforce that are able to detect and contain threats at the local or national level before these problematics become global crises.

8. Block positions (Isa)

8.1Affected countries : These countries experienced massive outbreaks, prioritizing urgent containment measures and international aid.

Guinea: Epicenter of the 2014 outbreak; declared rapid emergencies and coordinated WHO vaccination efforts for ongoing surveillance. Initial resistance due to cultural mistrust.
Liberia: Thousands of deaths in 2014–15; closed borders with Guinea and used military quarantines to achieve "zero cases."
Sierra Leone: Greater urban impact; deployed troops for safe burials and clinics following initial panic in Freetown.
Democratic Republic of Congo: Recurring outbreaks through 2025; militarized response in conflict zones using experimental WHO vaccines.
8.2 Partner Countries: Led the global response with funding, medical personnel, and logistics via the UN/WHO.
United States of America: Invested >$3 billion; the CDC and military built hospitals in West Africa.
United Kingdom: £140 million and clinics in Sierra Leone; DFID support for safe burials.

China: $80 million, 1,200 volunteers, and medical teams with no political strings attached.

Cuba: 256 doctors in Sierra Leone and Guinea; emphasis on free primary care.

Nigeria: Contained 5 imported cases in 2014; a regional success story thanks to rapid contact tracing.
8.3 Neutral Countries: Symbolic contributions, focused on self-protection without direct interventions.
Russia: Limited medical teams in Guinea; rejected alarmism and prioritized borders.

India: Minor donations to the WHO; internal surveillance without deployments due to distance.
Mexico: Preventive airport measures; zero cases and minimal regional aid.

Indonesia: Small funds and screening; neutral due to low exposure to Asia.

Saudi Arabia: Lower UN donations; screening for Hajj and pilgrims.

Switzerland: Funded the WHO from Geneva; vaccine expertise without on-the-ground presence.

9. Relevant Documents (Ari)

9.1 International Health Regulations (IHR 2005):

This is the primary legal framework that governs how countries must respond to health crises. It sets the rules for declaring a "Public Health Emergency of International Concern" (PHEIC).

WHO International Health Regulations (3rd Edition)

9.2 The WHO Constitution:

The foundational document that outlines the powers and the mission of the organization.

Constitution of the World Health Organization

9.3 UN Security Council Resolution 2177 (2014):

A historic document where the UN declared a disease outbreak as a threat to global security. It encourages countries to lift travel and trade restrictions that might hurt the economies of affected nations.

https://undocs.org/S/RES/2177(2014)

9.4 WHO Ebola Response Roadmap:

The specific strategic plan created to coordinate the global fight against the 2014 epidemic. It breaks down the response into "Pillars" like case management and social mobilization.

Ebola Response Roadmap

9.5 Resolution WHA69.9: Reform of WHO's Work in Emergencies:

A "lessons learned" document that explains how the WHO changed its structure after the Ebola crisis. It created the "WHO Health Emergencies Programme," which is how the agency responds to outbreaks today.

WHA69.9 Resolution

9.6 The 2014-2016 Ebola Outbreak Fact Sheet:

A great starting point for delegates who need a summary of the most famous epidemic. It provides the "Key Facts" that every delegate should have in their opening speech.

WHO Ebola Virus Disease Fact Sheet

10. Previous attempts to solve the problem

Since the 2014 West African outbreak, the world community's response to the Ebola virus has drastically changed. In the early phases of such a crisis, the global reaction was mostly scattered and reactive; despite this, many solutions have been proposed to solve this epidemic.

The creation of the United Nations Mission for Ebola Emergency Response (UNMEER) was one of the most significant early attempts to solve the problem. It manifested itself as a historical shift because it was the first time that the United Nations established a mission dedicated to a public health emergency. This mission aimed to provide high-level political leadership and logistical support to Guinea, Liberia, and Sierra Leone, whereas UNMEER helped coordinate the "70-70-60" target, by isolating 70% of cases and providing 70% of safe burials within 60 days of the mission. This reflected a critical stage; it reflected how, when a virus was detected, the world lacked a permanent, meaning that there was no standing medical force that could be deployed. ("UN Mission for Ebola Emergency Response (UNMEER)")

Whenthe epidemic in 2016 was concluding, the WHO's objective was to imporve these structural weaknesses through establishing the WHO Health Emergencies Programme (WHE) and the Contingency Fund for Emergencies (CFE). These actions were done and executed in order to make the organizations more operational rather than just advisory. For instance, the CFE was a landmark attempt to solve the "funding gap". It was achieved by permitting the WHO to release millions of dollars within 24 hours of an outbreak instead of waiting for international donations to arrive. This new system was put into action during the 2018–2020 outbreaks in the Democratic Republic of the Congo. As an international response, the use of "Ring Vaccination" and the deployment of advanced monoclonal antibody treatments was achieved. This response was successful in reducing death rates, yet it faced new adversities regarding security and community trust in active conflict zones. (WHO)

In recent years, the focus of the solutions has shifted towards Regional Empowerment and Permanent Preparedness. African nations collaborated to strengthen the Africa Centres for Disease Control and Prevention (Africa CDC) after they recognized that the Western-led responses were slow or culturally insensitive. Thus, the response or solutions in a way have "Africanized", meaning that it prioritized African needs and customs. This action led to better community engagement and faster local detection for these pathogens.

In addition, the global community has established initiatives like the Pandemic Fund as an attempt to solve the long-term issue of vaccine manufacturing. Initiatives such as these aimed to provide consistent financing to low-income countries in order to obtain laboratory upgrades. Even though many attempts to solve this problem have been done the international response fails to correctly address or solve once and for all the "panic and neglect" cycle. This occurs when the world offers enormous assistance during a crisis, yet neglects to keep up the required medical infrastructure after the Ebola threat has subsided. (AfricaCDC)

11. Committee objectives (Isa)

- Assess risks and issue temporary recommendations.
- Propose improvements in prevention and transparency.
- Stop transmission, treat cases, and maintain essential services.

12. Guiding questions

What can be done to prevent the epidemic from spreading?

How can the Ebola virus be prevented?

what the country has to offer economically

What measures did your government take in the first two weeks after the outbreak was detected?

How do different countries interpret and apply protections for civilians?

How can humanitarian agencies and international organizations improve practical support programs?

How can international cooperation be strengthened to ensure safe

13. Bibliography (Ari)

14. Aditional Resources (Ari)

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